

The Governed *Care Workflow*

One resident's journey through the whole system — from referral to reassessment — with the embedded decision rules that fire at each step.
The engine classifies; the score only advises; a human authorises anything that touches level or fee.

Rules classify · score is advisory

Banding **OFF** until calibrated GAP-001

L5 only via **authorised pathway**

Three separate engines · LOC ≠ PCG ≠ ACS

No event auto-changes **fee or level**

DRAFT until nurse approves

Emergency **never delayed** by revenue

Roles · **Care Manager** (senior clinical oversight) → **Nurse** → **Caregiver**

REFERENCE

The rule vocabulary

AS-01...14 + NS-01

14 scored assessment domains (0–4) plus the non-scored profile anchor.

MLR-001...019

19 Minimum-Level Rules — clinical floors that set the Level of Care.

MOD ×13

13 clinical modifiers — shape the plan & tasks; never inflate the score.

DT-001...014

14 decision-tree protocols (+132 atomic BR-* rules) for every event type.

CEG-01...07

Care-event engine rules — the exception / escalation matrix.

PCG-001...012

DT-013 Private-Caregiver rules — dedicated 1:1 staffing.

ACS-001...015

DT-014 Additional Clinical Services — discrete skilled add-ons.

END TO END

The process, stage by stage



Referral & admission suitability

CARE MANAGER · NURSE

A referral is reviewed against LifeCare's capability — can we safely meet this person's needs — guided by the DT-001 protocol. The Care Manager makes the call; DT-001 is the reference protocol, not an automated gate.

DT-001 · reference protocol

In referral / enquiry Out accepted → Stage 1, or declined / referred out

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Resident assessment (v4.2)

NURSE · CARE MANAGER

Each of the 14 domains is scored 0–4 against fixed anchors, with the evidence the anchor requires. The intrinsic need is scored regardless of who currently delivers the care.

AS-01 ADLs

AS-02 Mobility

AS-03 Falls

AS-04 Cognition

AS-06 Clinical

AS-11 Skin

...AS-14

NS-01 profile

In observed need + evidence Out domain scores · raw acuity /56

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Classification → Final Level of Care

ENGINE → CARE MANAGER / NURSE CONFIRM

Deterministic order: an authorised **L5 comfort pathway** supersedes; else a clinical override (never below its floor); else the highest of the 19 MLR floors. The raw /56 band is advisory only and is **not** auto-applied — the Care Manager or Nurse confirms the Final LOC. 13 modifiers are suggested for reconciliation.

max(MLR floors)

MLR-018 → L5

+ modifiers

DT-002

CAPABILITY GATE — L4/L5 & MLR-011/015/016/019 require confirming LifeCare can safely deliver before proceeding.

Out Final LOC L1–L5 + DT-013 / DT-014 *review recommendations only*

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Governance gate

CARE MANAGER / NURSE AUTHORISE

Before anything downstream applies, the guards run in code: a validated assessment is an authorised reassessment by the Care Manager or Nurse who signed it off.

NO AUTO-FEE — a level/fee change needs an explicit authoriser

feeOrLevelChangeAllowed

DOWNGRADE — a lower level requires a fresh, authorised reassessment

canLowerLevel

Out authorised → apply; otherwise blocked with the reason surfaced

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Apply the Final LOC

SYSTEM · IDEMPOTENT

The confirmed level fans out to the artefacts the rest of the app already consumes.

careLevel enum

LOC monthly fee

LOC history

L1 Independent · L2/L3 Assisted · L4 Memory · L5 Skilled/Palliative

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Care-plan generation

NURSE INDIVIDUALISES · CARE MANAGER APPROVES

A plan is generated from the locked Care-Task-Master text for the level's needs — created as a **DRAFT**. No tasks reach caregivers until the Nurse / Care Manager individualises it and **releases** it to active.

Care Task Master

status: DRAFT

DRAFT UNTIL RELEASED — release flips it to active; only then does the materialiser spin today's tasks.

Out individualised care plan + caregiver tasks

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Today's Care — shift delivery

CAREGIVER · NURSE

The active plan materialises into shift bundles the caregiver works through, charting **exception-first**: a one-tap complete for the expected, or a documented variance / change.

BND-AM

BND-BR

BND-MEAL

BND-MOB

BND-NOC

BND-RN

PACKAGE GATE — care outside the level's package warns and routes to DT-014 (pending determination) — it never auto-charges or blocks care.

Out a Care Event per delivered / varied task

Care events — the feedback loop

CAREGIVER CHARTS → ALERTS NURSE / CARE MANAGER

Every charted outcome is classified against the engine rules, which decide the escalation and run the variance counter. Acute events reach the emergency pathway; repeated variance raises a reassessment review — but nothing here ever changes the level or the fee automatically.

CEG-01...07

DT-003/004/011

DT-010 emergency

EMERGENCY PATHWAY — an unsafe event / acute deterioration routes to DT-010; escalation is never delayed or gated by revenue.

VARIANCE LOOP — 2+ material variances in the review window → nursing reassessment alert (no automatic LOC/fee change).

🔄 Reassessment (DT-012) feeds back to Stage 1 — the loop that keeps the level honest as the resident changes.



Reassessment

NURSE · CARE MANAGER

A change in condition, a repeat-variance review alert, or a care-plan review coming due prompts the Nurse or Care Manager to reopen the assessment — running the whole loop again and re-confirming (or changing) the Level of Care under the same governance. Reopening is a deliberate clinical action, guided by the DT-012 protocol (not an automatic scheduler).

DT-012 · reference protocol

→ Stage 1

INVARIANT Three separate pricing engines

ENGINE A · DT-002

Level of Care

The shared care package for L1–L5, billed as one monthly fee. This is the only engine driven by the assessment score/floors.

L1–L5 fee

ENGINE B · DT-013

Private caregiver

Dedicated 1:1 staffing beyond the shared package — night / 8h / 12h / 24h / temporary / elective. Mandatory only when shared staffing genuinely can't meet the need.

PCG-011 anti-double-charge BR-013.05

ENGINE C · DT-014

Additional clinical services

Discrete skilled add-ons that sit *outside* the LOC package. Change-of-condition monitoring is **not** an add-on — it's already in the level.

ACS-014 must be outside the package

Classification only recommends B and C for review — it never decides or charges them. Each engine runs independently, and no fee is posted twice for the same staffing rationale.

STAGE 7 DETAIL

Care-event escalation matrix

CHARTED OUTCOME	ENGINE RULE	ACTION	ROUTES TO	EMERGENCY
Completed / Not required	CEG-01	None — routine completion	—	—
Refused	—	Notify Nurse / Care Manager	DT-011	—
Unable	—	Plan review	DT-003	—
Unsafe	CEG-05	Incident + SBAR escalation	DT-004	DT-010
Increased assist	CEG-02	Plan review · variance +1	DT-012	—
Frequency variance	CEG-03	Plan review · variance +1	DT-012	—
Clinical change	CEG-05	Notify Nurse / Care Manager · acute	DT-003	DT-010
Repeated variance	CEG-06	Reassessment review alert	DT-012	—

2+ material variances in the review window raise a review alert — a human decides any change. No row auto-changes the level or the fee.

PROTOCOLS

The 14 decision trees

DT-001 Admission suitability	DT-002 Level of Care	DT-003 Change in condition
DT-004 Fall response	DT-005 Clinical escalation · SBAR	DT-006 Medication variance

DT-007 Safeguarding

DT-008 Infection / outbreak

DT-009 Hospital transfer

DT-010 Emergency / evacuation

DT-011 Care-task exception

DT-012 Care-level change

DT-013 Private caregiver

DT-014 Additional services

The guardrails that hold the whole loop together

Seven invariants enforced in code across every stage above.

- ✓ **Rules classify, score advises.** The raw /56 never sets the level on its own.
- ✓ **L5 is a pathway, not a score.** Only an authorised comfort/palliative pathway reaches it.
- ✓ **No event auto-changes fee or level.** Variance only raises a review; a human authorises change.
- ✓ **Emergency is never delayed** by revenue or capability — safety routes to DT-010 first.
- ✓ **Banding stays off** until the Care Manager / Nurse calibrate the 56-point thresholds.
- ✓ **Three engines stay separate.** LOC ≠ Private Caregiver ≠ Additional Services; no double-charge.
- ✓ **DRAFT until approved.** Generated plans dispatch nothing until the Nurse / Care Manager releases them.

LifeCare SLMS · governed clinical decision workflow

v4.2 assessment / v3.9 care model · 14 domains · 19 MLR · 14 DT · 3 engines